

Riverbend General Hospital — Neurology / Stroke Unit

Acute Stroke Evaluation Report (Fictional Test Data)

Patient Name:	Maria L. Sampleton	Patient ID:	TEST-000456
Date of Birth:	1958-07-22 (Age 68)	Sex:	Female
Date of Report:	2026-09-26	Referring Physician:	Dr. Amit Rao
Admission Date:	2026-09-26	Department:	Neurology / Stroke Unit

Chief Complaint

Sudden onset of right-sided weakness and slurred speech, witnessed onset 90 minutes prior to arrival.

History of Present Illness

Patient was found by family member with acute onset right facial droop, right arm and leg weakness, and difficulty speaking (slurred, word-finding difficulty). Last known well time documented. No head trauma. No seizure activity witnessed. Family activated emergency services immediately.

Past Medical History

Atrial fibrillation (not on anticoagulation), Hypertension, Hyperlipidemia, prior Transient Ischemic Attack (2023). Non-smoker. No prior stroke.

Vital Signs on Arrival

Blood Pressure	178/104 mmHg	Heart Rate	112 bpm (irregular)
Respiratory Rate	20 /min	Temperature	37.1 C
SpO2	95% on room air	Blood Glucose	142 mg/dL

Neurological Examination

NIHSS Score	14 (moderate-severe stroke)
Consciousness	Alert, oriented x2
Facial Droop	Right-sided, present
Arm Weakness	Right arm, 2/5 strength (drifts and falls)
Leg Weakness	Right leg, 3/5 strength
Speech	Dysarthria, mild expressive aphasia
Sensory	Decreased sensation, right side
Visual Fields	Right homonymous hemianopia suspected

Laboratory Results

Test	Result	Reference Range	Flag
Blood Glucose	142 mg/dL	70-99 mg/dL	HIGH
Platelet Count	245 x10 ⁹ /L	150-400 x10 ⁹ /L	Normal
INR	1.0	0.8-1.1	Normal
aPTT	28 sec	25-35 sec	Normal
Sodium	138 mmol/L	135-145 mmol/L	Normal
Creatinine	1.1 mg/dL	0.6-1.2 mg/dL	Normal
Troponin I	0.03 ng/mL	<0.04 ng/mL	Normal

Diagnostic Imaging

Non-contrast CT Head: No acute hemorrhage. Early ischemic changes noted in the left middle cerebral artery (MCA) territory. ASPECTS score 7.

CT Angiography: Occlusion of the left M1 segment of the middle cerebral artery.

CT Perfusion: Large area of salvageable penumbra with small ischemic core, favorable for intervention.

ECG: Atrial fibrillation with rapid ventricular response, rate 112 bpm.

Assessment / Diagnosis

1. Acute ischemic stroke, left MCA territory (M1 occlusion)
2. Atrial fibrillation with rapid ventricular response, not previously anticoagulated
3. Hypertensive urgency
4. History of prior TIA
5. NIHSS 14 — moderate-severe stroke

Plan / Recommendations

1. IV thrombolysis (alteplase) administered per protocol within therapeutic window
2. Urgent mechanical thrombectomy — interventional neuroradiology consulted
3. Admit to Stroke/Neuro ICU for close neurological monitoring
4. Blood pressure management per post-thrombolysis protocol
5. Cardiology consult for atrial fibrillation management and anticoagulation planning
6. Serial NIHSS assessments every hour for first 24 hours
7. Repeat CT head at 24 hours to assess for hemorrhagic transformation
8. Swallow assessment prior to oral intake
9. Early involvement of physical therapy, occupational therapy, and speech-language pathology
10. Family counseling regarding prognosis and rehabilitation planning

Attending Physician: Dr. Amit Rao, MD, Neurology

Report generated: 2026-09-26

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